

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase 1b/2 Open-Label Clinical Study to Evaluate the Safety and Efficacy of MK-6070 and Ifinatamab Deruxtecan (I-DXd) in Participants With Relapsed/Refractory Extensive-Stage Small Cell Lung Cancer
Short Title:	A Study to Evaluate the Safety and Efficacy of Gocatamig (MK-6070) and Ifinatamab Deruxtecan (I-DXd) in Participants With Relapsed/Refractory Extensive-Stage Small Cell Lung Cancer (MK-6070-002)
Protocol Number:	915516394246150130
NCT Number:	NCT06780137
Sponsor Name:	Merck
Investigational Product:	durvalumab
Phase:	PHASE1, PHASE2
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Number of Participants Who Experience an Adverse Event (AE): An AE is any untoward medical occurrence in a clinical study participant, temporally associated with the use of study intervention, whether or not considered related to the study intervention. An AE can therefore be any unfavorable and unintended sign (including an abnormal laboratory finding), symptom, or disease (new or exacerbated) temporally associated with the use of a study intervention. The number of participants who experience an AE in the study will be presented. 2. Number of Participants Who Experience One or More Dose-Limiting Toxicities (DLTs): A DLT is defined as any drug-related adverse event (AE) observed during the DLT evaluation period that meet pre-defined DTL criteria. Toxicities will be graded using National Cancer Institute Common Terminology for Adverse Events (NCI CTCAE) version 5.0, or the American Society for Transplant and Cellular Therapy (ASTCT) criteria. The number of participants who experience at least one DLT will be presented. 3. Number of Participants Who Discontinue Study Intervention Due to an AE: An AE is any untoward medical occurrence in a clinical study participant, temporally associated with the use of study intervention, whether or not considered related to the study intervention. An AE can therefore be any unfavorable and unintended sign (including an abnormal laboratory finding), symptom, or disease (new or exacerbated) temporally associated with the use of a study intervention. The number of participants who discontinue the study intervention due to an AE in the study will be presented.
Secondary Obj.:	1. Part 1, Part 2 (Arm 5 and Arm 6), and Part 3 (Arm 7): Duration of Response (DOR) 2. Part 1, Part 2 (Arm 6), and Part 3 (Arm 7): Progression-Free Survival (PFS) 3. Part 2

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(Arm 5 and Arm 6) and Part 3 (Arm 7): ORR

2.2 Background & Rationale

Researchers are looking for new ways to treat people with extensive-stage small cell lung cancer (SCLC) that has relapsed or is refractory. Gocatamig is a new type of immunotherapy that uses a person's immune system to find and destroy cancer cells. Ifinatamab deruxtecan (also known as I-DXd) is a drug which binds to a specific target on cancer cells and delivers treatment to destroy those cells. Durvalumab is a different type of immunotherapy that also destroys cancer cells. Researchers want to know if giving gocatamig, I-DXd, and gocatamig with I-DXd or durvalumab can treat SCLC that did not respond or stopped responding to a prior treatment.

3. Study Design & Methodology

Design Framework: Type: INTERVENTIONAL, Phase: PHASE1, PHASE2, Status: RECRUITING

Target N: 242

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Has histologically or cytologically confirmed SCLC that is extensive stage (defined as Stage IV (T any, N any, M1a/b/c) following at least 1 prior line of systemic therapy that included platinum-based chemotherapy
- * Must be able to provide archival tumor tissue sample or fresh biopsy tissue sample
- * Human immunodeficiency virus (HIV) infected participants must have well controlled HIV on antiretroviral therapy (ART)

4.2 Exclusion Criteria

- * Pleural effusion, pericardial effusion, or ascites requiring recurrent drainage procedure
- * History of (noninfectious) pneumonitis/interstitial lung disease (ILD) that required steroids or has current pneumonitis/ILD, and or suspected ILD/pneumonitis
- * Has clinically severe pulmonary compromise resulting from intercurrent pulmonary illnesses
- * Active or history of immune deficiency with the exception of HIV-infected participants with well controlled HIV on ART
- * History within 6 months before the first dose of study intervention of coronary/peripheral artery bypass graft and/or any coronary/peripheral angioplasty or clinically significant cardiovascular disease such as myocardial infarction, symptomatic congestive heart failure (CHF) (New York Heart Association > class II), and/or uncontrolled cardiac arrhythmia
- * History of arterial thrombosis (eg, stroke or transient ischemic attack) within 6 months before the first dose of study intervention
- * Active clinically significant infection requiring systemic therapy
- * History of allogeneic tissue/solid organ transplant
- * History of leptomeningeal disease
- * Received prior radiotherapy within 2 weeks of start of study intervention, or has radiation-related toxicities, requiring corticosteroids
- * Receiving chronic systemic steroid therapy (in dosing exceeding 10 mg daily of prednisone equivalent) or any other form of chronic immunosuppressive therapy within 7 days prior to the first dose of study intervention

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- * Known additional malignancy that is progressing or has required active treatment within the past 3 years
- * Untreated or symptomatic brain metastases
- * Active viral hepatitis, defined as hepatitis A (hepatitis A virus immunoglobulin M \[IgM\] positive in the setting of associated signs/symptoms), hepatitis B (hepatitis B virus surface antigen \[HbsAg\] positive and/or detectable hepatitis B virus \[HBV\] deoxyribonucleic acid \[DNA\]), or hepatitis C (hepatitis C virus \[HCV\] antibody positive and detectable HCV ribonucleic acid). Participants with HBV with undetectable viral load after treatment are eligible. Participants with HCV with undetectable virus after treatment are eligible.
- * Part 1 only: Radiation therapy to the lung ≥ 30 Gy within 6 months before the start of study intervention
- * Part 1 only: Abdominal radiation within 4 weeks before start of study intervention
- * Part 1 only: Anticancer hormonal treatment (except luteinizing hormone-releasing hormone \[LHRH\]) within 2 weeks before start of study intervention
- * Part 1 only: Systemic anticancer therapy (except antibody-based anticancer therapy) or investigational agents within 3 weeks or 5 half-lives, whichever is longer
- * Part 1 only: Antibody-based cancer therapy within 3 weeks before start of study intervention
- * Part 1 only: Chloroquine/hydroxychloroquine within 2 weeks before start of study intervention
- * Part 1 only: Clinically significant corneal disease
- * Part 1 only: Has other uncontrolled or significant protocol-specified cardiovascular disease

11. Study Identifiers & Governance

Primary ID: 915516394246150130

Registry Number: NCT06780137

Sponsor: Merck

Study Title: A Study to Evaluate the Safety and Efficacy of Gocatamig (MK-6070) and Ifinatamab Deruxtecan (I-DXd) in Participants With Relapsed/Refractory Extensive-Stage Small Cell Lung Cancer (MK-6070-002)

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____